

Local Coverage Determination (LCD)

Total Knee Arthroplasty

L36575

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Noridian Healthcare Solutions, LLC	A and B MAC	01111 - MAC A	J - E	California - Entire State
Noridian Healthcare Solutions, LLC	A and B MAC	01112 - MAC B	J - E	California - Northern
Noridian Healthcare Solutions, LLC	A and B MAC	01182 - MAC B	J - E	California - Southern
Noridian Healthcare Solutions, LLC	A and B MAC	01211 - MAC A	J - E	American Samoa Guam Hawaii Northern Mariana Islands
Noridian Healthcare Solutions, LLC	A and B MAC	01212 - MAC B	J - E	American Samoa Guam Hawaii

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
				Northern Mariana Islands
Noridian Healthcare Solutions, LLC	A and B MAC	01311 - MAC A	J - E	Nevada
Noridian Healthcare Solutions, LLC	A and B MAC	01312 - MAC B	J - E	Nevada
Noridian Healthcare Solutions, LLC	A and B MAC	01911 - MAC A	J - E	American Samoa California - Entire State Guam Hawaii Nevada Northern Mariana Islands
Noridian Healthcare Solutions, LLC	A and B MAC	02101 - MAC A	J - F	Alaska
Noridian Healthcare Solutions, LLC	A and B MAC	02102 - MAC B	J - F	Alaska
Noridian Healthcare Solutions, LLC	A and B MAC	02201 - MAC A	J - F	Idaho

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Noridian Healthcare Solutions, LLC	A and B MAC	02202 - MAC B	J - F	Idaho
Noridian Healthcare Solutions, LLC	A and B MAC	02301 - MAC A	J - F	Oregon
Noridian Healthcare Solutions, LLC	A and B MAC	02302 - MAC B	J - F	Oregon
Noridian Healthcare Solutions, LLC	A and B MAC	02401 - MAC A	J - F	Washington
Noridian Healthcare Solutions, LLC	A and B MAC	02402 - MAC B	J - F	Washington
Noridian Healthcare Solutions, LLC	A and B MAC	03101 - MAC A	J - F	Arizona
Noridian Healthcare Solutions, LLC	A and B MAC	03102 - MAC B	J - F	Arizona
Noridian Healthcare Solutions, LLC	A and B MAC	03201 - MAC A	J - F	Montana

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Noridian Healthcare Solutions, LLC	A and B MAC	03202 - MAC B	J - F	Montana
Noridian Healthcare Solutions, LLC	A and B MAC	03301 - MAC A	J - F	North Dakota
Noridian Healthcare Solutions, LLC	A and B MAC	03302 - MAC B	J - F	North Dakota
Noridian Healthcare Solutions, LLC	A and B MAC	03401 - MAC A	J - F	South Dakota
Noridian Healthcare Solutions, LLC	A and B MAC	03402 - MAC B	J - F	South Dakota
Noridian Healthcare Solutions, LLC	A and B MAC	03501 - MAC A	J - F	Utah
Noridian Healthcare Solutions, LLC	A and B MAC	03502 - MAC B	J - F	Utah
Noridian Healthcare Solutions, LLC	A and B MAC	03601 - MAC A	J - F	Wyoming

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Noridian Healthcare Solutions, LLC	A and B MAC	03602 - MAC B	J - F	Wyoming

LCD Information

Document Information

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L36575

LCD Title

[Total Knee Arthroplasty](#)

Proposed LCD in Comment Period

N/A

Source Proposed LCD

[DL36575](#) 

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For services performed on or after 09/07/2016

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N/A

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N/A

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07/22/2016

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09/06/2016

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Issue

Issue Description

This update is to consolidate JE and JF to have one unified document and policy number.

CMS National Coverage Policy

Title XVIII of the Social Security Act, §1862 (a)(1)(A) allows coverage and payment for only those services that are considered to be reasonable and necessary for the diagnosis or treatment of illness or injury or to improve the functioning of a malformed body member.

Title 21, Code of Federal Regulations, Chapter 1, Subchapter H, Part 888 Orthopedic Devices, Section 888.1100 Arthroscope

Title 42, Code of Federal Regulations, §482.24

CMS Manual System, Pub 100-08, *Medicare Program Integrity Manual*, Chapter 6, §6.5.2

CMS Manual System, Pub 100-08, *Medicare Program Integrity Manual*, Chapter 3, §3.4.1.3

Coverage Guidance

Coverage Indications, Limitations, and/or Medical Necessity

Joint replacement surgery has been performed on millions of people over the past several decades and has proved to be an important medical advancement in the field of orthopedic surgery. The hip and knee are the two most commonly replaced joints. The knee is the largest joint in the body and includes the lower end of the femur, the upper end of the tibia and the patella. The knee joint has three compartments, the medial, the lateral and the patellofemoral. The surfaces of these compartments are covered with articular cartilage and are bathed in synovial fluid. The bones of the knee joint work together, allowing the knee to function smoothly.

The most common reason for total knee replacement surgery is arthritis of the knee joint. Types of arthritis include:

- osteoarthritis,
- rheumatoid arthritis and
- traumatic arthritis (arthritis which occurs as a result of injury).

Arthritis causes a severe limitation in the activities of daily living (ADLs), including difficulty with walking, squatting, and climbing stairs. Pain is typically most severe with activity and patients often have difficulty getting mobilized when seated for a long time. Other findings include chronic knee inflammation or swelling not relieved by rest, knee stiffness, lack of pain relief after taking non-steroidal anti-inflammatory medications and failure to achieve symptom improvement with other conservative therapies such as steroid injections and physical therapy.

Osteonecrosis and malignancy are additional reasons to proceed with total knee replacement surgery. The use of TKR in patients with malignancy must be weighed against considerations of life expectancy and possible alternative procedures to relieve pain. The goal of total knee replacement surgery is to relieve pain and improve or increase patient function.

Occasionally, there may be a need to perform a reoperation on a previous total knee replacement. This is often referred to as a revision total knee.

Circumstances that lead to the need for a revision total knee are continued disabling pain, continued decline in function which can be attributed to failure of the primary joint replacement. Failure can be due to infection involving the joint, substantial bone loss in the structures supporting the prosthesis, fracture, aseptic loosening of the components and wear of the prosthetic components.

TOTAL KNEE ARTHROPLASTY (TKA)

Indications:

Noridian will consider total knee replacement surgery medically necessary in the following circumstances:

Advanced joint disease demonstrated by:

- Radiographic supported evidence or when conventional radiography is not adequate, magnetic resonance imaging (MRI) and/or computed tomography (CT) (in situations when MRI is non-diagnostic or not able to be performed) supported evidence (subchondral cysts, subchondral sclerosis, periarticular osteophytes, joint subluxation, joint space narrowing, avascular necrosis); **and**
- Pain or functional disability from injury due to trauma or arthritis of the joint; **and**
- If appropriate, history of unsuccessful conservative therapy (non-surgical medical management) that is clearly addressed in the pre-procedure medical record. **(If conservative therapy is not appropriate, the medical record must clearly document why such approach is not reasonable)**. Non-surgical medical management is usually but not always implemented prior to scheduling total joint surgery. Non-surgical treatment as clinically appropriate for the patient's current episode of care typically includes one or more of the following:
 - anti-inflammatory medications or analgesics, or
 - flexibility and muscle strengthening exercises, or
 - supervised physical therapy [Activities of daily living (ADLs) diminished despite completing a plan of care], or
 - assistive device use, or
 - weight reduction as appropriate, or
 - therapeutic injections into the knee as appropriate.

In some circumstances, for example, if the patient has bone on bone articulation, severe deformity, pain or significant disabling interference with activities of daily living, the surgeon may determine that nonsurgical medical management would be ineffective or

counterproductive, and that the best treatment option, after explaining the risks, is surgical. If medical management is deemed inappropriate, the medical record should indicate the rationale for and circumstances under which this is the case.

- Failure of a previous osteotomy; or
- Distal femur fracture; or
- Malignancy of the distal femur, proximal tibia, knee joint or adjacent soft tissues; or
- Failure of previous unicompartmental knee replacement; or
- Avascular necrosis of the knee; or
- Proximal tibia fracture

*See the associated Billing and Coding article (linked below) for Documentation Requirements.

Indications for Replacement/Revision of [Total Knee Arthroplasty](#)

- Loosening of one or more components, or
- Fracture or mechanical failure of one or more component, or
- Infection, or
- Treatment of periprosthetic fracture of distal femur, proximal tibia or patella, or
- Progressive or substantial periprosthetic bone loss, or
- Bearing surface wear leading to symptomatic synovitis, or
- Implant or knee misalignment, or
- Knee stiffness/arthrofibrosis, or
- Tibiofemoral instability, or
- Extensor mechanism instability

Limitations

Noridian will not consider a total knee replacement medically necessary when the following contraindications are present:

- Active infection of the knee joint or active systemic bacteremia
- Active urinary tract or dental infection
- Active skin infection (exception recurrent cutaneous staph infections) or open wound within the planned surgical site of the knee
- Rapidly progressive neurological disease

The following conditions are relative contraindications to total knee replacement and if such surgery is performed in the presence of these conditions, it is expected that the rationale for proceeding with the surgery under such circumstances is clearly documented in the medical record:

- Insufficiency of extensor mechanism/quadriceps
- Any process that is rapidly destroying bone
- Neurotrophic arthritis

This local coverage determination (LCD) is only addressing medical necessity criteria for performing total knee replacement surgery. With respect to knee replacement surgery, there is a form of knee joint replacement surgery called unicompartmental knee replacement. This is typically done for patients with osteoarthritis of the knee in which the damage is contained to one compartment of the knee. The indications outlined in this LCD are not to be applied for unicompartmental knee replacement surgery. Failed previous unicompartmental joint replacement is an indication for performing a **total knee arthroplasty**.

Summary of Evidence

N/A

Analysis of Evidence (Rationale for Determination)

N/A

General Information

Associated Information

Utilization Guidelines

It is expected that these services would be performed as indicated by current medical literature and/or standards of practice. When services are performed in excess of established parameters they may be subject to review for medical necessity.

The devices/implants utilized for total knee replacement surgeries are regulated by the FDA as medical devices. The devices used should be class II or class III devices that meet the requirements outlined in CFR 21, Chapter 1, subchapter H, Part 888.

Sources of Information

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11.Your Orthopedic Connection (2010). Unicompartmental knee replacement. Retrieved from <http://orthoinfo.aaos.org/topic.cfm?topic=A00585>. Please note that this reference is not endorsed as official guidelines from the AAOS.

Bibliography
N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
11/06/2025	R4	Revision Effective Date: 11/06/2025 CONTRACTOR INFORMATION: Added: JF contractor information This update is to consolidate JE and JF to have one unified document and policy number. Pursuant to the 21st Century Cures Act, this revision does not require notice and comment because this update is considered non-substantive	Other (Change in Affiliated Contract Numbers)

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
		and does not alter the intent of coverage or non-coverage outlined in any LCD.	
12/01/2019	R3	The LCD is revised to remove CPT/HCPCS codes in the Keyword Section of the LCD. At this time 21st Century Cures Act will apply to new and revised LCDs that restrict coverage which requires comment and notice. This revision is not a restriction to the coverage determination; and, therefore not all the fields included on the LCD are applicable as noted in this policy.	Other (The LCD is revised to remove CPT/HCPCS codes in the Keyword Section of the LCD.)
12/01/2019	R2	As required by CR 10901, all billing and coding information has been moved to the companion article; this article is linked to the LCD. At this time 21st Century Cures Act will apply to new and revised LCDs that restrict coverage which requires comment and notice. This revision is not a restriction to the coverage determination; and, therefore not all the fields included on the LCD are	Revisions Due To Code Removal

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
		applicable as noted in this policy.	
10/01/2016	R1	Effective 09/07/2016 LCD revised to remove the following 7th character information from Paragraph 1 and added each appropriate 7th character to each of the appropriate diagnosis codes. The appropriate 7th character is to be added to each code from category M80 as well as to each code from subcategories M84.3, M84.4, M84.5 and M84.6 from the following list: - o A: initial encounter for fracture - o D: subsequent encounter for fracture with routine healing - o G: subsequent encounter for fracture with delayed healing - o K: subsequent encounter for fracture with nonunion - o P: subsequent encounter for fracture with malunion - o S: sequela The appropriate 7th character is to be added to all of the codes from category S72 from the following list: - o A: initial encounter for closed	• Other (Added all the appropriate 7th characters to all the appropriate DX codes.) • Revisions Due To ICD-10-CM Code Changes

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
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fracture

- o B: initial encounter for open fracture type I or II initial encounter for open fracture NOS
- o C: initial encounter for open fracture type IIIA, IIIB, or IIIC
- o D: subsequent encounter for closed fracture with routine healing
- o E: subsequent encounter for open fracture type I or II with routine healing
- o F: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with routine healing
- o G: subsequent encounter for closed fracture with delayed healing
- o H: subsequent encounter for open fracture type I or II with delayed healing
- o J: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with delayed healing
- o K: subsequent encounter for closed fracture with nonunion
- o M: subsequent encounter for open fracture type I or II with nonunion
- o N: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with nonunion
- o P: subsequent encounter for closed fracture with malunion
- o Q: subsequent encounter for

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
		open fracture type I or II with malunion - o R: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with malunion - o S: sequela EXCEPT to subcategory S72.47 to which the appropriate 7th character is to be added to all codes from the following list: - o A: initial encounter for closed fracture - o D: subsequent encounter for closed fracture with routine healing - o G: subsequent encounter for closed fracture with delayed healing - o K: subsequent encounter for closed fracture with nonunion - o P: subsequent encounter for closed fracture with malunion - o S: sequela	
		The appropriate 7th character is to be added to each code from subcategory S79.1 from the following list: - o A: initial encounter for closed fracture - o D: subsequent encounter for fracture with routine healing - o G: subsequent encounter for fracture with delayed healing - o K: subsequent encounter for	

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
		fracture with nonunion o P: subsequent encounter for fracture with malunion o S: sequela	
		The appropriate 7th character is to be added to all of the codes from category S82 from the following list: o A: initial encounter for closed fracture o B: initial encounter for open fracture type I or II initial encounter for open fracture NOS o C: initial encounter for open fracture type IIIA, IIIB, or IIIC o D: subsequent encounter for closed fracture with routine healing o E: subsequent encounter for open fracture type I or II with routine healing o F: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with routine healing o G: subsequent encounter for closed fracture with delayed healing o H: subsequent encounter for open fracture type I or II with delayed healing o J: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with delayed healing o K: subsequent encounter for	

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
		closed fracture with nonunion - o M: subsequent encounter for open fracture type I or II with nonunion - o N: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with nonunion - o P: subsequent encounter for closed fracture with malunion - o Q: subsequent encounter for open fracture type I or II with malunion - o R: subsequent encounter for open fracture type IIIA, IIIB, or IIIC with malunion - o S: sequela EXCEPT to subcategory S82.16 to which the appropriate 7th character is to be added to all codes from the following list: - o A: initial encounter for closed fracture - o D: subsequent encounter for closed fracture with routine healing - o G: subsequent encounter for closed fracture with delayed healing - o K: subsequent encounter for closed fracture with nonunion - o P: subsequent encounter for closed fracture with malunion - o S: sequela	
		The appropriate 7th character	

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
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is to be added to each code from subcategory S89.0 from the following list:

- o A: initial encounter for closed fracture
- o D: subsequent encounter for closed fracture with routine healing
- o G: subsequent encounter for closed fracture with delayed healing
- o K: subsequent encounter for closed fracture with nonunion
- o P: subsequent encounter for closed fracture with malunion
- o S: sequela

The appropriate 7th character is to be added to each code from category T84 from the following list:

- o A: initial encounter
- o D: subsequent encounter
- o S: sequela

Effective 10/01/2016 LCD revised to add ICD-10-CM codes M97.111XA, M97.11XD, M97.11XS, M97.12XA, M97.12XD & M97.12XS.

Effective 10/01/2016 deleted ICD-10 CM codes T84.042A, T84.042D, T84.042S, T87.043A, T84.043D &

Revision History Date	Revision History Number	Revision History Explanation	Reasons for Change
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T84.0431S

Associated Documents

Attachments

N/A

Related Local Coverage Documents

Articles

- A57685 - Billing and Coding: Total Knee Arthroplasty
- A55077 - Response to Comments: Total Knee Arthroplasty
- A55079 - Response to Comments: Total Knee Arthroplasty

LCDs

- DL36575 - Total Knee Arthroplasty (MCD Archive Site)

Related National Coverage Documents

NCDs

N/A

Public Versions

Updated On	Effective Dates	Status	
10/27/2025	11/06/2025 - N/A	Currently in Effect	You are here
01/29/2020	12/01/2019 - 11/05/2025	Superseded	View
11/19/2019	12/01/2019 - N/A	Superseded	View

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Keywords

- **TOTAL KNEE ARTHROPLASTY**
- Total knee
- arthroplasty